BASELINE → TREATMENT → FOLLOW-UP

# Treatment checks across the course

Relative stages, not equally spaced dates. No patient dates are needed; agree who arranges and reviews checks.

| Check | Before starting | Induction Day 1 | Induction Days 2–21 | Maintenance review | After treatment |
|---|---|---|---|---|---|
| **Bloods and hormones** | FBC, liver/kidney, glucose, thyroid, hepatitis B/C and GFR baseline | Review results and current symptoms | FBC, liver/kidney and glucose each cycle; thyroid every 3–6 weeks | Continue cycle bloods and thyroid checks | Follow unresolved problems and the team’s plan |
| **Treatment and routes** | Agree IV/SC atezolizumab and IV versus IV/oral etoposide | All three medicines in the source sequence | Etoposide Days 2–3; next cycle usually after 21 days | Carboplatin and etoposide stop after up to four cycles | Agree follow-up and next treatment decisions |
| **Support medicines** | Prepare one reconciled day-by-day anti-sickness plan | Carboplatin-specific high-risk cover | Account for etoposide Days 2–3 and two days afterwards | No automatic continuation of chemotherapy prophylaxis | Continue only individually prescribed medicines |
| **Immune effects and cancer review** | Baseline breathing, bowel, skin and endocrine symptoms; alert card | Report administration reactions immediately | Report new cough, diarrhoea, rash or unusual weakness promptly | Review benefit and symptoms; scans per consultant | Delayed immune effects remain possible |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00689 v4 · official regimen

THE WHOLE TREATMENT JOURNEY

# Atezolizumab + carboplatin + etoposide: treatment and checks

### First-line extensive-stage small-cell lung cancer

Induction: atezolizumab and carboplatin on Day 1, with etoposide on Days 1–3, repeated every 21 days for up to four cycles. Maintenance: carboplatin and etoposide stop; atezolizumab alone continues every 21 days while appropriate. The fourth cycle ends about 12 weeks after starting if there are no delays; maintenance has no fixed total duration in this regimen.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Induction Day 1 | Induction Days 2–21 | Maintenance review | After treatment |
|---|---|---|---|---|---|
| **Bloods and hormones** NCCP 00689 | FBC, liver/kidney, glucose, thyroid, hepatitis B/C and GFR baseline | Review results and current symptoms | FBC, liver/kidney and glucose each cycle; thyroid every 3–6 weeks | Continue cycle bloods and thyroid checks | Follow unresolved problems and the team’s plan |
| **Treatment and routes** NCCP 00689 | Agree IV/SC atezolizumab and IV versus IV/oral etoposide | All three medicines in the source sequence | Etoposide Days 2–3; next cycle usually after 21 days | Carboplatin and etoposide stop after up to four cycles | Agree follow-up and next treatment decisions |
| **Support medicines** NCCP 00689 | Prepare one reconciled day-by-day anti-sickness plan | Carboplatin-specific high-risk cover | Account for etoposide Days 2–3 and two days afterwards | No automatic continuation of chemotherapy prophylaxis | Continue only individually prescribed medicines |
| **Immune effects and cancer review** NCCP 00689 | Baseline breathing, bowel, skin and endocrine symptoms; alert card | Report administration reactions immediately | Report new cough, diarrhoea, rash or unusual weakness promptly | Review benefit and symptoms; scans per consultant | Delayed immune effects remain possible |

# What each check is for

1 Before treatment

Before first dose

Confirm suitability, phase and routes

**Ask / act:** Review results, contraindications, medicines and explicit route prescriptions.

Sources: NCCP 00689

2 Phase change

After up to four induction cycles

Chemotherapy ends; immune treatment may continue

**Ask / act:** Confirm atezolizumab schedule and revise anti-sickness medicines without stopping required hormone replacement.

Sources: NCCP 00689

3 After treatment

Completion or change of plan

Delayed effects and follow-up

**Ask / act:** Give follow-up arrangements, alert-card advice and urgent contact details.

Sources: NCCP 00689

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-26 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.23